

KOTAMIN TABLET 4MG

DESCRIPTION:

A 7mm round, scored tablet, yellow colour with marking 'duo'.

COMPOSITION:

Each tablet contains Chlorpheniramine Maleate 4mg.

PHARMACODYNAMICS:

Antihistamine. KOTAMIN is a member of the alkylamine group of antihistamines, which are among the most active H₁ blockers. They are effective in low dosage, have a wide safety margin, and a very low incidence of side effects such as dizziness and drowsiness.

PHARMACOKINETICS:

Chlorpheniramine maleate is absorbed relatively slowly from the gastro-intestinal tract, peak plasma concentrations occurring about 2.5 to 6 hours after administration by mouth. Bioavailability is low, values of 25 to 50% having been reported. Chlorpheniramine appears to undergo considerable first pass metabolism. About 70% of chlorpheniramine in the circulation is bound to plasma proteins. Chlorpheniramine is widely distributed in the body, including passage into the CNS. Chlorpheniramine maleate is extensively metabolized. Metabolites include desmethyl- and didesmethylchlorpheniramine. Unchanged drug and metabolites are excreted primarily in the urine; excretion is dependent on urinary pH and flow rate. Only trace amounts have been found in the faeces. Duration of action of 4 to 6 hours has been reported; this is shorter than may be predicted from pharmacokinetic parameters. More rapid and extensive absorption, faster clearance, and a shorter half-life have been reported in children.

INDICATION:

For all conditions requiring antihistamine therapy such as hay fever, vasomotor rhinitis, urticaria, angioneurotic oedema, food allergies, drug and serum reactions, atopic and contact dermatitis, allergic eczema, pruritus ani et vulvae, insect bites.

RECOMMENDED DOSAGE:

Tablet: Adults – One tablet, up to four times daily as required.
Children – 6 - 12years: 2-4mg t.d.s or q.i.d.

ROUTE OF ADMINISTRATION: Oral

CONTRAINDICATIONS:

Hypersensitivity to chlorpheniramine. Traditional antihistamines such as chlorpheniramine is contraindicated in patients taking an antihypertensive agent or an antidepressant medication containing a monoamine oxidase inhibitor MAOI.

WARNINGS & PRECAUTIONS:

WARNINGS: Alcohol potentiates the action of antihistamines, and drowsiness may occur in patients not otherwise subject to it if alcohol is not taken.

PRECAUTIONS:

May cause drowsiness. Patients should be warned against engaging in mechanical operations which require alertness, such as driving a motor vehicle, until response to the drug has been determined. Should be used with caution in patients with narrow angle glaucoma, stenotic peptic ulcer, pyloroduodenal obstruction and urinary bladder obstruction due to symptomatic prostatic hypertrophy and narrowing of the bladder neck.

Might have the sedative effect potentiated by concomitant use with alcohol, tricyclic antidepressants, barbiturates or other CNS depressants. MAOI's prolong and intensify the effects of antihistamines. The action of oral anticoagulants may be inhibited by antihistamines.

Are more likely to cause dizziness, sedation and hypotension in elderly patients (approximately 60 years and older).

Paediatric Use. In infants and children especially, antihistamines in overdosage may cause hallucinations, convulsion or death. Antihistamines in young children particularly may cause excitation.

Antihistamines should not be used to treat lower respiratory tract symptoms, asthma especially.

Elderly patients (60 years or older). More likely to cause dizziness, sedation and hypotension. May precipitate fits in epileptics with large doses of antihistamines.

INTERACTION WITH OTHER MEDICAMENTS:

Chlorpheniramine with Alcohol – CNS depressant effects of chlorpheniramine are enhanced

Chlorpheniramine with Barbiturates, Bromvaletone, Carbromal, Chloral hydrate, Dichloralphenazone, Glutethimide hydrate, Magnesium glutamate hydrobromide, Nitrazepam, Triclofos – Chlorpheniramine enhances the CNS depressant effects of the hypnotic sedatives listed.

Chlorpheniramine with Phenytoin – Serious adverse effects when chlorpheniramine given with Phenytoin. Plasma levels of phenytoin increased. (Single case report)

USE IN PREGNANCY & LACTATION:

Unnecessary administration of drugs during the first trimester of pregnancy is undesirable.

SIDE EFFECTS:

Drowsiness and dizziness are rare, but patients should be warned that if they occur, they should not drive a car or operate a machine.

CNS depression

- Fatigue, inability to concentrate, insomnia, irritability, tremor, nightmares, hallucinations and euphoria have occasionally been reported. In overt intoxication CNS stimulation may lead to convulsions.

- Vertigo, tinnitus, blurred vision, diplopia, dilated pupils, labyrinthitis, dry mouth, nose and throat, due to an anticholinergic effect

Haematological – blood dyscrasias, agranulocytosis, haemolytic anemias, leucopenia, thrombocytopenia, although rare has been reported.

Genitourinary – voiding difficulties, dysuria, frequency retention.

Respiratory – thickening of bronchial secretions, tightness in chest, wheezing, nasal stuffiness

General – hypothermia, drug rash, anaphylactic shock, photo-sensitivity, excessive perspiration.

Cardiovascular – hypotension, tachycardia, hypertension, headache, palpitations, ECG changes. Overdosage can lead to cardiovascular collapse.

SYMPTOMS AND TREATMENT OF OVERDOSE:

In the event of overdosage, emergency treatment should be started immediately. Treatment of the signs and symptoms of overdosage is symptomatic and supportive. Stimulants should not be used.

STORAGE CONDITIONS:

Store below 30°C. Protect from light. Keep out of reach of children. *Jauhkan daripada kanak-kanak.*

SHELF LIFE:

Please refer to outer package.

PACK SIZE:

100 x 10's, 500 x 10's and 10 x 10's in blister pack.

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD

Lot 2599, Jalan Seruling 59 Kawasan 3,

Taman Klang Jaya, 41200 Klang, Selangor, MALAYSIA